

OBSTETRIC CASE RECORD

Name : Mrs. Bhargavi

OP / IP No.

Age : 24 Sex : F

Date of Admission :

Address : Egmore

Date of discharge :

Ward No. :

Socio Economic Status : Class IV

Diagnosis : Education : 10th Std

Occupation : House wife

Aadhar No. :

Mobile No. :

Obstetric Score : G₂P₁L₁

LMP : 10.3.21

EDD : 17.12.21

CHIEF COMPLAINTS:

Patient come with 9 months of amenorrhoea for safe confinement of pregnancy.

History of presenting illness :

No H/o abdominal pain

No H/o bleeding or discharging PV

No H/o pedal edema

Menstrual History :

Normal cycle , 3/30

No H/o pain. No H/o passing clots

Attained menarche at 12 years of age

Marital History :

Married at 18 years of age

Non consanguineous marriage

First child born after 1 year

Obstetric History :

Previous pregnancy : Conceived at 19 years. 1st, 2nd and 3rd trimester uneventful. NVD at term in ~~10~~ 11. Female baby - 2.5 kg ; cried immediately after birth ; No postpartum complications exclusively breastfed for 6 months ; Baby is alive and healthy.

Present pregnancy: Booked and immunized at ~~1st~~ 2nd month, 2 doses of Td given at 1st and 5th month, Antenatal visits - 6.

I trimester:

Pregnancy confirmed by Urine pregnancy Test at RSKH at 4 weeks of missed period. No H/O vomiting; No H/O fever with chills. No H/O radiation exposure. No H/O drug intake. Folic acid tablets were taken. Dating scan done at 3rd month and was found to be normal.

Family History:

II trimester:

Quickening felt at 5th month; No H/O bleeding PV. No H/O pedal edema, headache, blurring of vision; No H/O polyuria, polydipsia; IFA-tablets taken; calcium tablets taken. Anomaly scan done at 5th month and was found to be normal.

Personal History:

III trimester:

Fetal movements perceived; No bleeding / draining PV. USG done at 7th and 9th month and were found to be normal.

Treatment History:

Past history:

No H/O DM, HT, TB, asthma, epilepsy, Thyroid disease. No H/O previous surgeries; No H/O blood transfusions.

GENERAL PHYSICAL EXAMINATION:

Personal history:

Mixed diet, Bowel and bladder habits normal, No adverse social habits.

General Examination:

Moderately built and nourished

No pallor, No icterus, No pedal edema

Cvs - S₁, S₂ heard no murmurs

RS - Normal vesicular breath sounds, No added sounds

CNS - No Focal Neurological deficit.

Breast, thyroid, spine and gait - Normal.

VITALS :

PULSE : 87/min BP : 120/80 mm Hg

RESP. RATE : 14/min

TEMP : Afebrile

Ht : 152 cm

Wt : 56 kg

BMI : 24.24

SYSTEMIC EXAMINATION :

INSPECTION : Uterus longitudinally enlarged.
Umbilicus in midline.
Linea nigra, stria gravidarum seen.
All quadrants move equally with respiration.
Flanks full on sitting position.

PALPATION : Fundal height corresponds to 32 weeks of gestation. ✓ Flanks full.
Symphysiofundal height - 33 cm
Abdominal girth - 102 cms.

FUNDAL GRIP : Broad, soft, bulky, not independently ballotable mass felt, suggestive of breech occupying the fundus.

UMBILICAL GRIP :

LEFT SIDE : smooth, curved uniform resistance felt, suggestive of spine.

RIGHT SIDE : multiple small nodule like structures felt, suggestive of limb buds.

FIRST PELVIC GRIP :

Hard round, independently ballotable occupying the lower pole of uterus, suggestive of foetal head.

AUSCULTATION : FHS heard in Lt. spinoumbilical line ; Rate - ~~140~~ 140/min
Normal rhythm.

DIAGNOSIS:

A 24 year old female, Mrs. Bhargavi G₂P₁L₁ with LMP on 10.3.21 and EDD on 17.3.21, who on examination has a single live fetus in cephalic presentation with good fetal heart sound has been admitted for safe confinement of pregnancy.

INVESTIGATIONS:

complete hemogram
Blood urea, sugar
serum creatinine
Urine albumin, sugar
Blood grouping, Rh typing
VDRL, HIV, HbsAg
ECG, Echo, Chest X ray
USG Abdomen
LFT, RFT, TFT.

TREATMENT:

DISCHARGE AND FOLLOW UP ADVICE